

المجلس الطبي المتخصص

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Name: سعاد عبدالرؤف عبدالرازق عيسى National ID: 26707092200447	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

ماد
ر.ب. الزرقا ج. الزاوي ج.د
قرية الدمنيم
مركز فاصل - نفي سوهاج



٢٦٧٠٧٠٩٢٢٠٠٤٤٧

KE6314996

DMS
مصلحة ج. الزاوي



٢٠٢٣/٠٩ ٢٦٧٠٧٠٩٢٢٠٠٤٤٧

ربة منزل

متزوجة

مسلمة

أنثى

جمال زكريا محمود البنا

البطاقة سارية حتى ٢٠٢٣/٠٩/٢٣





Pathology Report

Name: سعاد عبد الرؤوف عبد الرازق

Age: 50 years.

Clinical data: Right breast mass.

Specimen type: Tru cut biopsy.

Date: 10/12/2019

Lab. Number: 1371

Hospital Number: مبادرة/2019

استلم

Gross Description:

Two partly fragmented greyish white tissue cores, the longest was; 1.6 cm, totally submitted.

Microscopic Examination:

Histopathological examination of section prepared breast tissue core, infiltrated by invasive malignant growth, formed of cords and trabeculae of moderately anaplastic ductal epithelial cells, with desmoplastic stroma.

Diagnosis:

Invasive mammary carcinoma GII, of terminal duct unit, for IPT;
(ER, PR, Her2 & Ki67).

Prof. Dr. Lubna Omar

Prof. Dr. Essam Ayad

Name: سعاد عبد الروؤف عبد الرازق		
MR. : 1476		
Detailed Medical Report		
		السيد
تجبة طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient was diagnosed with triple-negative breast cancer (CT3, no metastatic disease, ER8/8, PR8/8, HER2-negative) and received neoadjuvant chemotherapy followed by radiation therapy and breast-conserving surgery in June 2020. Adjuvant tamoxifen was initiated and subsequently switched to letrozole (Femara) in November 2022 in conjunction with zoledronic acid (Zometa). The patient presented for a Femara refill and was noted to have dyspnea; a chest CT was ordered, which demonstrated right basal subpleural mass (R BSM) and pleural effusion (PAUS). The imaging results are pending. Follow-up imaging in July 2026 showed a normal mammogram, a calcified gallbladder with an atrophic kidney and a uterine fibroid on ultrasound, and a goiter with calcification on chest CT. The patient is scheduled for a neck ultrasound and a follow-up visit for Femara on 2 November 2026.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
	الطبيب المعالج : dina makram	
يعتمد ,		



Patient Name: Souad Abdalrwaf Abdel Razek Owais	Date : 23/03/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 55 Year(s)
Medical ID:	Pat. ID: 11893

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

I Chest:-

- Right upper lobe anterior segment sub-pleural reticulations likely post therapeutic sequel.
- Small right upper lobe fissural based nodule is seen measuring about 5 mm.
- No otherwise sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Relatively small sized left kidney is noted showing no stones or back-pressure changes.
- Normal size, parenchymal thickness & density of the right kidney with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



Patient Name: Souad Abdalrwaf Abdel Razek Owais	Date : 23/03/2023
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 55 Year(s)
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- Bulky uterus with anterior wall fibroid is seen measuring about 5.5 cm.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.
- No lytic or sclerotic lesions.

Opinion:

Known case of breast cancer showing:

- *Small right upper lobe fissural based nodule likely of benign nature.*
- *Bulky uterus with anterior wall fibroid*

Dr. Ahmed M. Rozeka, MSc

Signature Prof. Menna Tallah Hanafy
27/03/2023 03:37 م



وحدة زلازل الأورام

قسم الدثولرجى

القصر العلى

وحدة تشخيص امراض الثدى

IMMUNOHISTOCHEMISTRY REPORT

Patient Name: سعاد عبد الرعوف عبد الرازق

lab.No:2020-27/ (4TM)

Date: 1 /2020

Sex: Female

CLINICAL DATA: A case of breast cancer, for estrogen, progesterone receptors and Her-2/neu.

PROCEDURE:

Sections were prepared from paraffin blocks treated for (ER clone SP1), (PR clone 112), (Her 2 neu-clone 4B5) automatically stained in Bench mark ET (VENTANA) using UL TRA VIEW DAB Detection Kit, Hematoxylin as counter stain.

RESULTS:

Examination of sections treated for ER and PR receptors revealed intense nuclear staining in most of tumor cells by chromogen.

Examination of sections treated for Her-2/neu revealed no membranous staining of tumor cells by chromogen.

CONCLUSION:

Tumor Cells Were:

- ✓ Positive For ER & PR, Score 8/8.
- ✓ Negative For Her-2/neu, (Score 0).

Comment: All controls show appropriate reactivity. The H&E slide as well as paraffin block were re-enclosed back.

PROF. DR. AHMED ABD AL-AZIZ

PROF. DR. SAMIA GABAL

أ.د. أحمد عبد العزيز
أ.د. سامية جبال



Exam Information:

<u>Patient Name:</u> SOAD A	<u>Date of birth:</u> 09/07/1967
<u>Arabic Name:</u> سعاد عبد الزوروف	<u>Patient Gender:</u> F
<u>Patient RIS ID:</u> 323609	<u>Exam Date:</u> 30/03/2023
<u>Patient HIS ID:</u> 123006054	<u>Approved by:</u>
<u>Written by:</u>	Dr. Hoda Fathy
Dr. Hoda Fathy	

^{99m}Tc MDP Bone Scan Report (26-3-2023)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of ^{99m}Tc MDP
Revealed:

- No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
- Degenerative changes are noted over the dorso-lumbar vertebrae.
- Arthritic changes are noted over both shoulder and both knee joints.
- Both kidneys are visualized.

Conclusion:

- No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Dr.Omnia Talaat,MD

Much Obligated